

SALES & MARKETING PLAN

Cellmax New Phase

August-December 2026

Commercial direction

Build a predictable digital-to-appointment pathway and progress towards RM100,000 in monthly clinic revenue by December 2026.

THE FIVE-MONTH ARC

AUG	Foundation
SEP	Awareness
OCT	Referrals
NOV	Diabetic wound campaign
DEC	Scale & retention

THE NEW COMMERCIAL PHASE

Registration opens the door; the operating system creates growth

Cellmax registration removes a major barrier to market entry. It does not replace the need for accurate claims, approved advertising, clinical assessment or disciplined conversion.

RM100,000

monthly clinic revenue direction
by December 2026

01

Clinical governance

Registered indications, patient assessment and appropriate approvals.

02

Measurable demand

Source tracking, qualified leads, appointments and treatment-plan starts.

03

Repeatable execution

Content cadence, WhatsApp follow-up and referral development.

THE STRATEGIC PRINCIPLE

Market a complete wound-care service — not a stand-alone vial.

Cellmax is one option within selected care plans following clinical assessment. The patient proposition is structured assessment, appropriate wound care, documented progress, education and scheduled follow-up.

Suitability depends on clinical assessment.

OBJECTIVES AND INTENDED OUTCOMES

Five outcomes define success

01 Position Klinik Inocare as a trusted, structured centre for chronic wound care.

02 Build a reliable path from digital discovery to WhatsApp, assessment and treatment-plan start.

03 Use Cellmax as a clinical differentiator governed by registered indications and patient assessment.

04 Increase referrals from clinics, home-care nurses, dialysis centres, care homes and communities.

05 Progress measurably towards RM100,000 in monthly clinic revenue by December 2026.

POSITIONING, AUDIENCES AND OFFER

A focused audience meets a complete service offer

Priority audiences

- 2.1

Patients and families managing diabetic, pressure, post-surgical or traumatic wounds.
- 2.2

Caregivers, home-care nurses, care homes and elderly-care providers.
- 2.3

Primary-care doctors, dialysis centres, pharmacies and diabetes organisations.
- 2.4

Kuala Lumpur and nearby Selangor areas suitable for repeat follow-up.

Service offer

- 2.5

Clinical wound assessment and measurement.
- 2.6

Progress documentation with patient consent.
- 2.7

Appropriate dressing selection and care planning.
- 2.8

Cellmax suitability assessment based on the patient's condition.
- 2.9

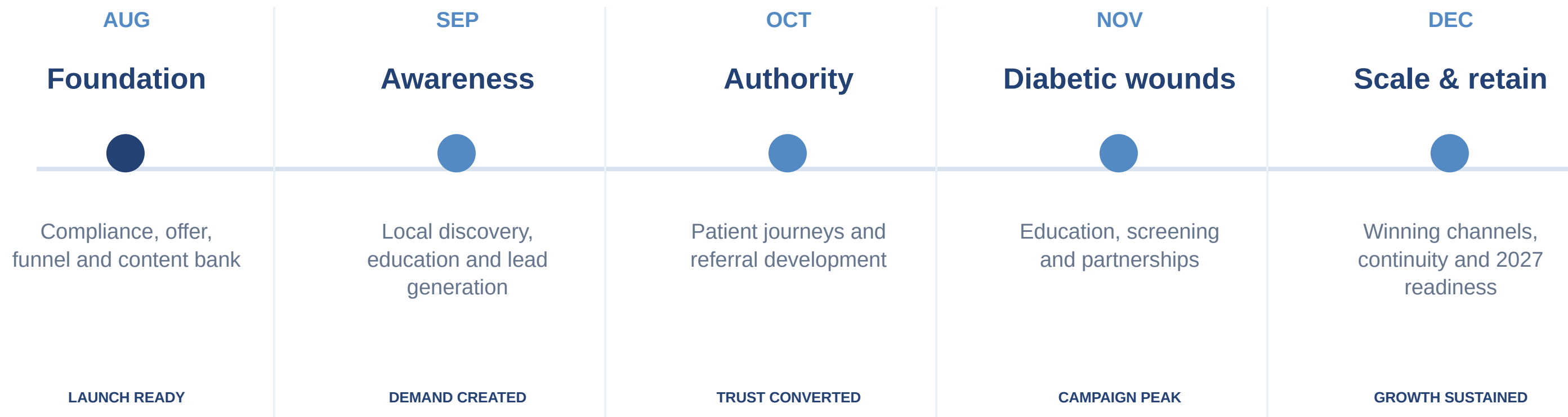
Scheduled follow-up, caregiver education and progress monitoring.

Core positioning

Comprehensive wound assessment and care, with Cellmax used only within selected plans after clinical assessment.

AUGUST-DECEMBER ROADMAP

Each month builds the next capability



The sequence prevents premature scaling: build the system first, generate demand second, deepen trust and referrals third, then scale what works.

AUGUST

2026

Foundation and
soft launch

CAMPAIGN THEME

A New Phase in Wound
Care at Klinik Inocare

Execution: 15-31 August

Build the operational, compliance and sales foundation before
scaling.

- 01

Verify the registration certificate, permitted claims, authorisation and ad-approval status.
- 02

Finalise package inclusions, pricing, payment options, capacity and weekly assessment slots.
- 03

Launch the wound-care landing page and direct WhatsApp call to action.
- 04

Set quick replies, screening, lead labels and mandatory source tracking.
- 05

Batch-produce six short videos and four posters or carousels; reactivate earlier enquiries.
- 06

Test two or three ad variations and prepare a concise professional referral kit.

Success markers

Compliance sign-off • Source tracking live • 6 videos + 4 creatives • 2-3 ad tests

SEPTEMBER

2026

Awareness and
lead generation

CAMPAIGN THEME

A Wound for More Than 2
Weeks? Do Not Wait.

Create consistent local demand and convert enquiries into booked assessments.

- 01

Publish three short videos and one educational carousel weekly, supported by near-daily Stories.
- 02

Focus education on diabetic, pressure, post-surgical and traumatic wounds.
- 03

Run Kuala Lumpur and Selangor Meta Ads, supported by Google Search or Maps.
- 04

Conduct two screening or community outreach activities.
- 05

Approach at least 20 organisations and build five active referral partners.
- 06

Use WhatsApp follow-up on Days 0, 1, 3 and 7 for prospects who have not booked.

Success markers

60 qualified leads • 24 assessments booked • 5 active referral partners • 2 outreach activities

OCTOBER

2026

Clinical authority
and referral
network

CAMPAIGN THEME

The Wound-Care Journey

Increase trust, attendance and conversion through transparent care processes.

- 01

Publish consented, anonymised patient-care journeys without guaranteed outcome language.
- 02

Produce doctor-led videos on assessment, delayed healing and the need for follow-up.
- 03

Retarget video viewers, landing-page visitors and WhatsApp starters.
- 04

Deliver concise clinical briefings to clinics, nurses, dialysis centres and care homes.
- 05

Create a simple referral route with a dedicated contact, short form and protected slots.
- 06

Collect Google reviews about service experience without unverified clinical claims.

Success markers

90 qualified leads • 36 assessments booked • 8 active referral partners • Referral route live

NOVEMBER

2026

Diabetic wound
campaign

CAMPAIGN THEME

Diabetes and Foot
Wounds: Act Before It Is
Too Late.

Use Diabetes Awareness Month to drive education, screening
and conversion.

- 01

Run a diabetic foot and wound screening programme with a direct route to assessment.
- 02

Publish focused content on foot care, warning signs, prevention and when to seek care.
- 03

Build educational partnerships with pharmacies, dialysis centres and diabetes groups.
- 04

Retarget unconverted September-October leads and reactivate discontinued follow-up.
- 05

Consider phased packages or payment options only when approved and clinically appropriate.
- 06

Use service-experience testimonials; avoid cure guarantees or misleading comparisons.

Success markers

120 qualified leads • 48 assessments booked • 16 plans started • 12 active partners

DECEMBER

2026

Scale, retention
and 2027
readiness

CAMPAIGN THEME

*Do Not Delay Wound
Care During the Holidays.*

Scale proven channels while protecting continuity of care and January demand.

- 01

Increase spend only on ads that generate qualified leads, appointments and plan starts.
- 02

Reactivate undecided prospects and secure January appointments before the break.
- 03

Prepare dressing and follow-up continuity plans for the holiday period.
- 04

Run satisfaction surveys and review clinical outcomes for August-November cases.
- 05

Recognise referral partners professionally without commissions or clinical inducements.
- 06

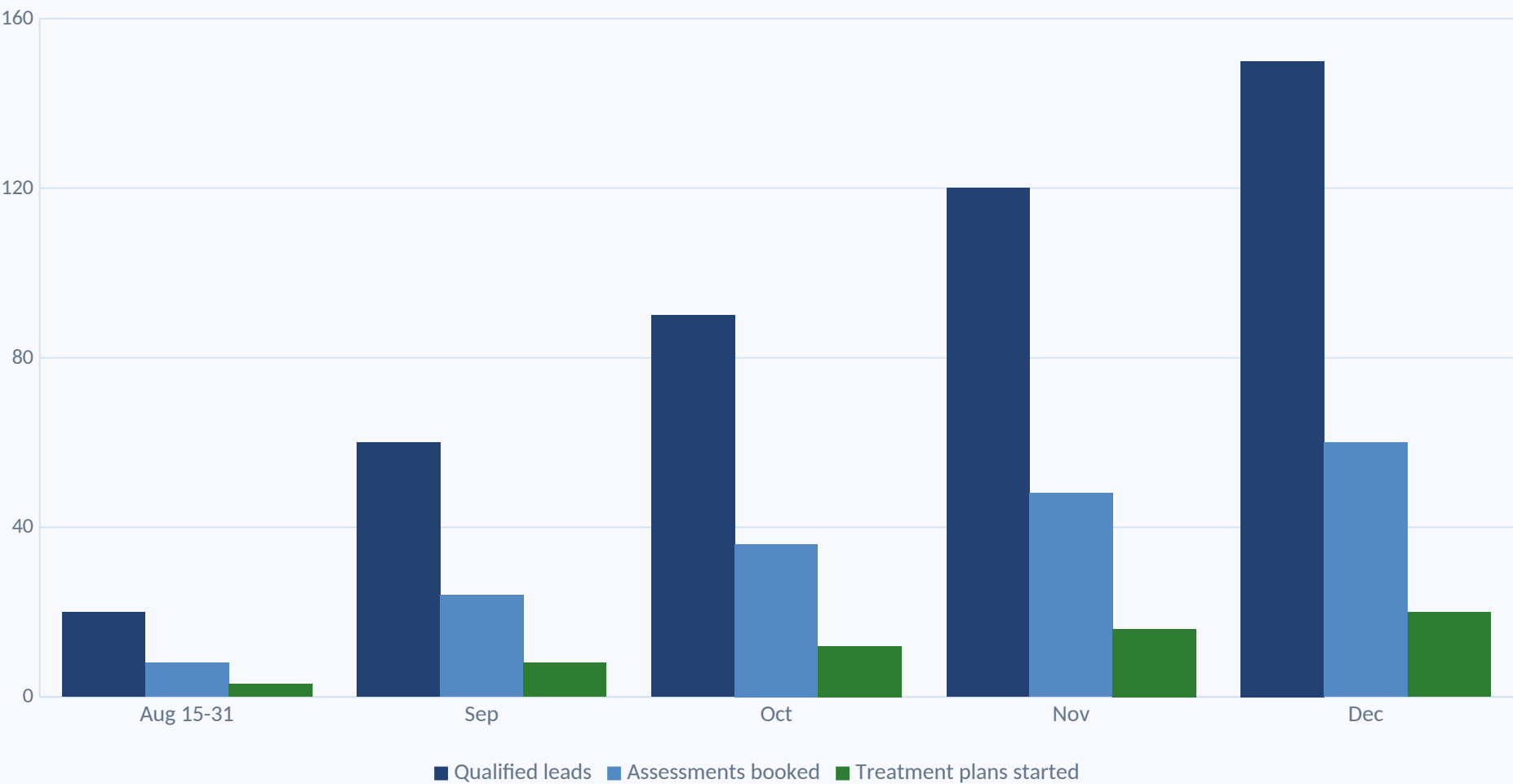
Present the August-December report and agree the January-June 2027 growth plan.

Success markers

150 qualified leads • 60 assessments booked • 20 plans started • RM100k direction

FUNNEL AND REVENUE TARGETS

Stretch targets create a measurable conversion funnel



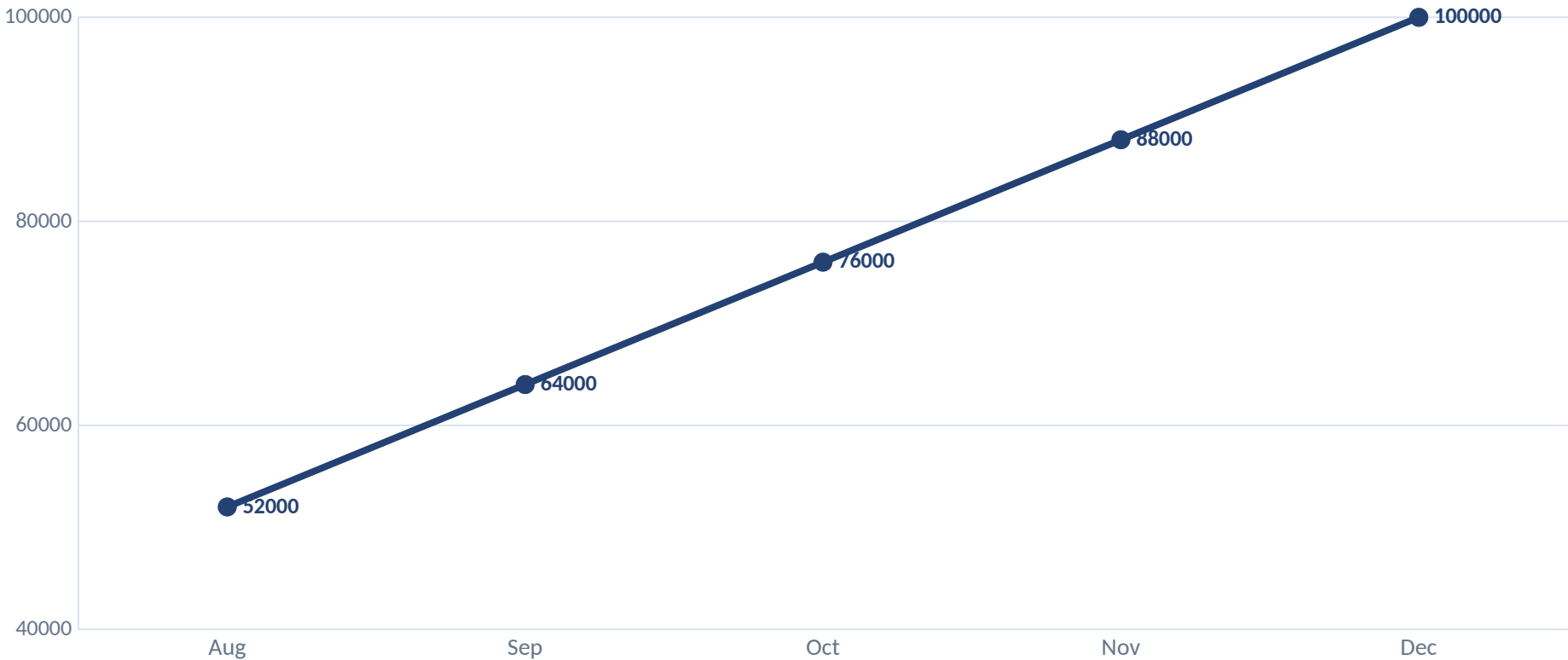
ACTIVE REFERRAL PARTNERS	
AUG	2
SEP	5
OCT	8
NOV	12
DEC	15

These are initial stretch targets. Calibrate them against clinic capacity, package pricing, supply and the July baseline.

PROPOSED CLINIC REVENUE RAMP

Close the revenue gap in controlled stages

B = actual July revenue baseline
M = RM100,000 December target



SHARE OF GAP CLOSED	
AUG	20%
SEP	40%
OCT	60%
NOV	80%
DEC	100%
Target formula: B + share × (M – B)	

WEEKLY CONTROL KPIS

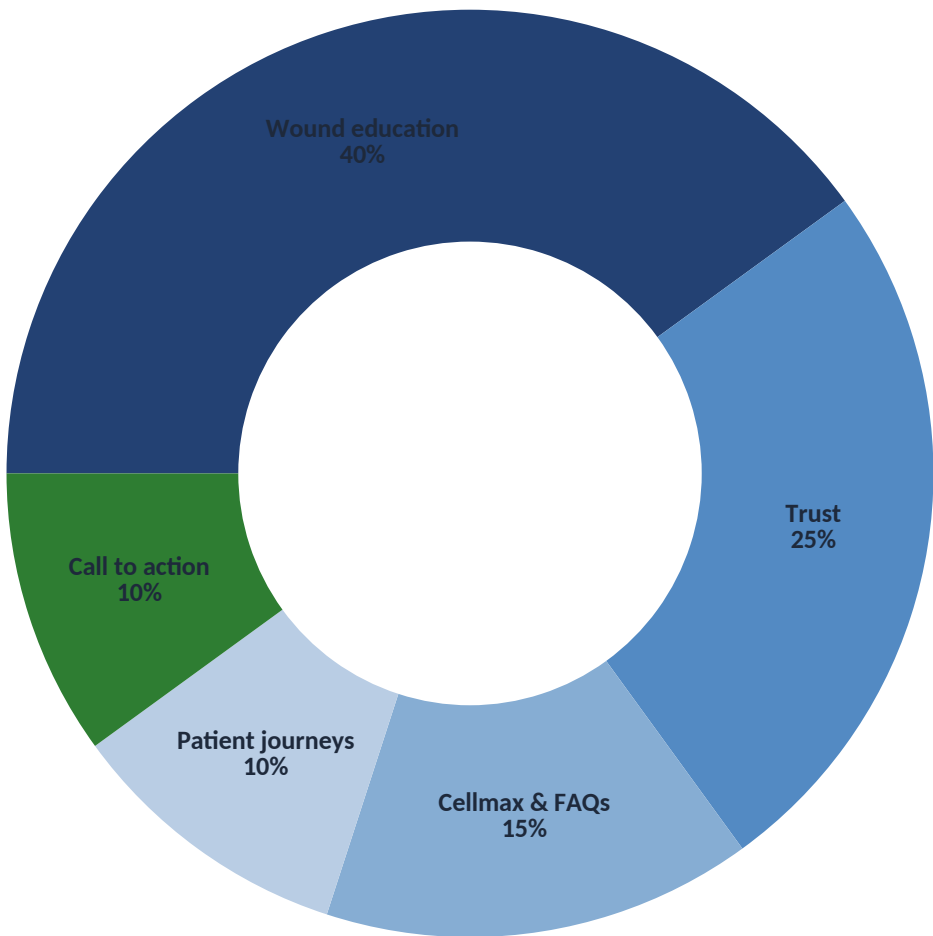
The weekly meeting should answer six questions

4.1 Demand How many qualified leads arrived, and from which sources?	4.2 Efficiency What are cost per lead, appointment and acquired patient?	4.3 Conversion How are booking, show-up and assessment-to-treatment rates moving?
4.4 Revenue Which source, service and campaign is producing revenue?	4.5 Responsiveness How quickly is WhatsApp answered, and is follow-up completed?	4.6 Referrals How many active partners and referred patients are being generated?

Initial operating targets: human response within 10 minutes during operating hours; ≥75% show-up rate; every prospect has a recorded source and follow-up status.

CONTENT AND CHANNEL STRATEGY

Education earns attention; trust and follow-up convert it



Primary channels

5.6	Facebook & Instagram	Families, caregivers and retargeting
5.7	TikTok	Short-form education and organic reach
5.8	Google	Business Profile, Search and high-intent landing-page demand
5.9	WhatsApp	Conversion, appointments and follow-up
5.10	Outreach & referrals	Demand beyond social media

Minimum cadence: 3 short videos + 1 carousel weekly, near-daily Stories, and one batch-recording session every two weeks.

SALES OPERATING SYSTEM

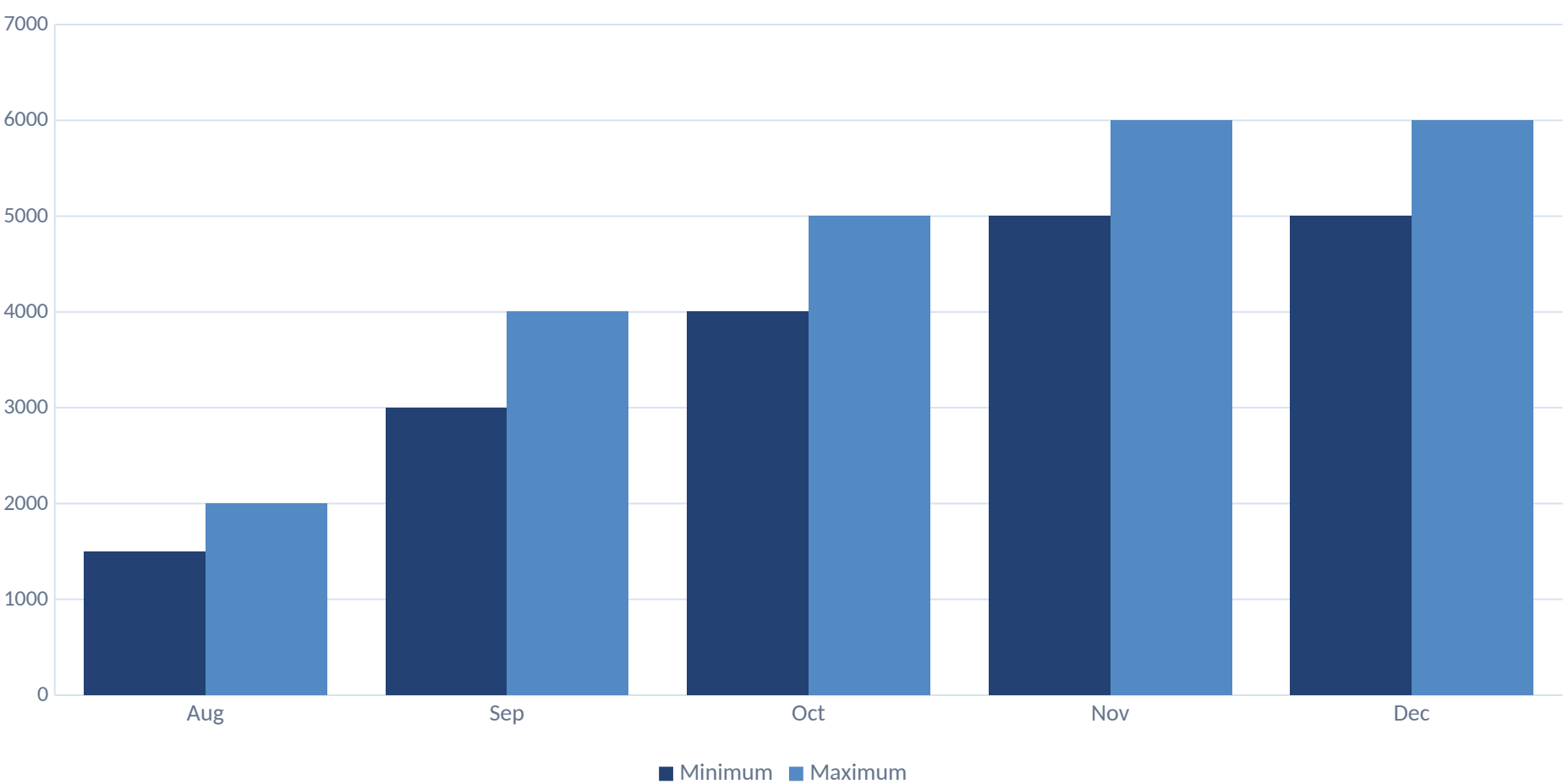
Every lead follows the same six-step service pathway

1	6.1	Record	Name, location, wound type, duration, contact details and source.
2	6.2	Respond	Explain image-assessment limits and conduct basic screening.
3	6.3	Book	Offer an appointment within 24-48 hours and send a reminder.
4	6.4	Explain	After assessment, clarify plan, price, duration, alternatives and expectations.
5	6.5	Follow up	Contact undecided prospects on Days 1, 3 and 7 without inappropriate pressure.
6	6.6	Monitor	Track care, adherence, service experience and clinical outcomes.

Conversion is a process, not a single reply.

INDICATIVE PAID-MEDIA BUDGET

Increase paid media only after lead quality is proven



TOTAL RANGE

RM18,500-
RM23,000

Paid media only

Scaling rule

Increase spend gradually only when qualified-lead quality, appointment cost and conversion are acceptable.

Production, outreach, printing and event-management costs require separate approval.

ROLES AND ACCOUNTABILITY

Clear ownership keeps the plan moving every week

01	Director / Clinical Lead	Approve clinical claims, medical content and treatment protocols.
02	Dr Johan	Coordinate campaigns, the performance dashboard, partnerships and management reporting.
03	Clinic Assistant	Manage WhatsApp, appointments, reminders, follow-up and lead-status updates.
04	Doctors / Nurses	Provide education, case documentation and clinical explanations.
05	Designer / Editor	Produce approved posters, videos, landing-page assets and advertising variations.

Weekly management rhythm

30 minutes to review leads, appointments, conversion, revenue, budget and operational issues.

COMPLIANCE AND RISK CONTROLS

Registration is not automatic approval for every advertisement

Important Every public poster, video, script and claim still needs the applicable approval path and authorised artwork.

- 9.1

Confirm registration number GD8996426-239437 against the current certificate.
- 9.2

Follow the applicable MDA approval route and use approved artwork or scripts.
- 9.3

Display Act 737, registration and advertisement-approval details when required.
- 9.4

Obtain written manufacturer or authorised-representative permission.
- 9.5

Review clinic-service advertising against MAB requirements before publication.
- 9.6

Avoid cure guarantees, superlatives, risk-free claims or unsupported comparisons.
- 9.7

Use written consent for patient images or testimonials; protect identity and health data.
- 9.8

Recheck approval when language, claims or visual messages change materially.

References: Medical Device Authority advertisement guidance; MDA/GL/04; Ministry of Health Pharmaceutical Services advertising FAQ.

DECISIONS REQUIRED

Six approvals unlock the soft launch

10.1 Monthly revenue targets and average treatment-plan value

10.2 Pricing, package inclusions, payment options and weekly capacity

10.3 August-December paid-media budget and scaling limits

10.4 MDA/MAB status, advertising authorisation and approver for each asset

10.5 Owners for content, WhatsApp, partnerships, reporting and clinical approval

10.6 Soft-launch date and first performance-review date

Approve the operating system first. Then scale only the channels that produce qualified patients, booked assessments and clinically appropriate care plans.